

1. Administrative & Study Identification

Full Study Title: EPIK-0: A Phase III, Multi-center, Randomized (1:1), Open-label, Active-controlled, Study to Assess the Efficacy and Safety of Alpelisib (BYL719) in Combination With Olaparib as Compared to Single Agent Cytotoxic Chemotherapy, in Participants With no Germline BRCA Mutation Detected, Platinum-resistant or Refractory, High-grade Serous Ovarian Cancer **Short Title/Acronym:** EPIK-0 / ENGOT-OV61 **Protocol Number:** CBYL719K12301 **NCT Number:** NCT04729387 **Posting ID:** 1051353345532287569 **Sponsor/ Company Name:** Novartis **Investigational Product:** Alpelisib (BYL719) in combination with Olaparib **Phase of Development:** Phase III **Trial Status:** Active, not currently recruiting (ACTIVE_NOT_RECRUITING) **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of the combination of alpelisib and olaparib compared with single agent cytotoxic chemotherapy (investigator's choice of paclitaxel or pegylated liposomal doxorubicin) in patients with platinum-resistant or refractory high-grade serous ovarian cancer, with no germline BRCA mutation detected. **Secondary Objectives:** To evaluate Overall survival, Number of participants with dose interruptions and dose reductions, and Dose intensity.

2.2 Background & Rationale Patients with platinum-resistant or -refractory high-grade serous ovarian cancer (HGSOC) have a poor prognosis, and their management represents a substantial unmet medical need. Preclinical data and results from a phase Ib trial demonstrated the efficacy and tolerability of the combination of the α -specific phosphatidylinositol-3-kinase (PI3K) inhibitor alpelisib plus the poly(ADP-ribose) polymerase (PARP) inhibitor olaparib in platinum-resistant, non-BRCA-mutated ovarian cancer, providing a robust rationale to investigate this combination against standard-of-care chemotherapy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Single agent cytotoxic chemotherapy: Paclitaxel or Pegylated liposomal Doxorubicin) **Blinding:** Open-label **Randomization:** 1:1

3.2 Planned Enrollment & Duration Target \$\$\$: 358 participants
Number of Sites: Multicenter (Global) **Estimated Study Start:** 02-Jul-2021 **Estimated Primary Completion:** 21-Apr-2023

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult women (≥ 18 years) with histologically confirmed diagnosis of high-grade serous or high-grade endometrioid ovarian cancer, fallopian tube cancer, or primary peritoneal cancer. 2. Measurable disease, i.e., at least one measurable lesion per RECIST 1.1 criteria, or if no measurable disease is present, disease should be assessable by Gynecologic Cancer Intergroup criteria (GCIC) for CA-125. 3. No germline BRCA1/2 mutation as determined by an FDA approved assay, with platinum-resistant or refractory disease, having had at least one but no more than three previous systemic therapies. 4. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 5. Adequate bone marrow and organ function.

4.2 Exclusion Criteria 1. Prior treatment with any PI3K, mTOR, or AKT inhibitor. 2. State of small or large bowel obstruction, or other impairment of gastrointestinal (GI) function/disease. 3. Liver impairment and Child Pugh score B or C. 4. Concurrently using other anti-cancer therapy. 5. Surgery within 14 days prior to starting study drug or has not recovered from major side effects. 6. Received radiotherapy ≤ 4 weeks or limited field radiation for palliation ≤ 2 weeks prior to randomization.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Alpelisib plus Olaparib administered continually until disease progression, unacceptable toxicity that precludes further treatment, or discontinuation due to any other reason. **Route of Administration:** Oral

5.2 Active Comparator Regimen (Chemotherapy) Agents: Single agent cytotoxic chemotherapy (Paclitaxel or Pegylated liposomal Doxorubicin). **Doxorubicin Drug Details:** ATC Code: L01DB01; Trade names include LipoDox, Caelyx, Doxil, Adriamycin; EMA reference: [Caelyx EPAR](#).

5.3 Concomitant & Prohibited Therapy Permitted: Standard supportive care. **Prohibited:** Concurrent use of other anti-cancer therapy.

6. Study Timeline & Visit Schedule

Visit ID	Window	Key Activities/Procedures	:---	:---
:---	Screening	2 to 4 weeks prior	Informed Consent,	

BRCA status testing, Baseline tumor imaging, CA-125 | | **Baseline** | Day 0 | Randomization, First Dose | | **Interim** | Every 28-day cycle | Safety Labs, Efficacy Assessment (RECIST 1.1) | | **Post-Treatment** | Varies | Safety follow-up visit and a 9-week post-progression visit | | **Survival Follow-up** | Varies | Overall survival monitoring |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression Free Survival (PFS) based on Blinded Independent Review Committee (BIRC) assessment using RECIST 1.1 criteria. Defined as the time from the date of randomization to the date of the first documented progression or death due to any cause. If a participant has not had an event, PFS will be censored at the date of the last adequate tumor assessment.

7.2 Secondary Outcomes 1. Overall survival 2. Number of participants with dose interruptions and dose reductions 3. Dose intensity

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Assessed and graded using the Common Terminology Criteria for Adverse Events (NCI CTCAE Version 4.03). Participants must have recovered from all toxicities related to prior therapies to baseline or $\leq$ Grade 1 (exception for alopecia). **Serious Adverse Events (SAEs):** Monitored continuously throughout treatment and the safety follow-up period. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee monitored safety and efficacy, including interim futility analyses.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for all primary and secondary efficacy analyses (all randomly assigned patients). The Safety Set included all patients who received ≥ 1 dose of study treatment. **Sample Size Justification:** Target enrollment of 358 participants, powered to detect a statistically significant improvement in PFS. **Handling of Missing Data:** Participants without an event were censored at the date of their last adequate tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) guidelines and the Declaration of

Helsinki. **Monitoring Plan:** Standard multicenter oncology clinical trial monitoring. **Audit Trail:** The trial protocol and all amendments were approved by the respective institutional review boards. All patients provided written informed consent.

11. Study Identifiers & Governance

Primary ID: CBYL719K12301 **Registry Number:** NCT04729387 **Posting ID:** 1051353345532287569 **Sponsor/Lead Organization:** Novartis **Study Title:** Alpelisib Plus Olaparib in Platinum-resistant/Refractory, High-grade Serous Ovarian Cancer, With no Germline BRCA Mutation Detected **Official Regulatory Title:** EPIK-0: A Phase III, Multi-center, Randomized (1:1), Open-label, Active-controlled, Study to Assess the Efficacy and Safety of Alpelisib (BYL719) in Combination With Olaparib as Compared to Single Agent Cytotoxic Chemotherapy, in Participants With no Germline BRCA Mutation Detected, Platinum-resistant or Refractory, High-grade Serous Ovarian Cancer **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting)

12. Clinical Framework (Ovarian Cancer Specific)

Disease Area / Primary Condition: Ovarian Cancer (Preferred UMLS: High-grade Serous Ovarian Cancer) **Diagnosis Threshold:** Histologically confirmed HGSOC, germline BRCA-wt, platinum-resistant or refractory **Medical Methodology:** PI3K inhibitor + PARP inhibitor combination targeted therapy vs Cytotoxic Chemotherapy (doxorubicin/paclitaxel) **Optimization Target:** Progression-Free Survival (PFS) prolongation

13. Study Procedures & Methodology

Management Pathway: Standard oncology evaluation for platinum-resistant/refractory disease **Education Protocol (HCP):** Site initiation and standardized application of RECIST 1.1 / GCIC CA-125 criteria **Education Protocol (Patient):** Toxicity self-monitoring **Quality Audit Mechanism:** Independent central imaging review via Blinded Independent Review Committee (BIRC)

14. Observation & Follow-Up Schedule

Onsite Visit Cadence: Evaluated in 28-day cycles **Remote Monitoring Frequency:** Continuous monitoring through the survival follow-up phase **Checklist Review Interval:** 9-week post-progression follow-up

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					